

FREE RESOURCE

5 Scripts for the Hard Conversations

What to say when your patient hasn't done their exercises,
is afraid to move, frustrated, comparing, or wants to rest

Plus: HEP Adherence Cheat Sheet

FROM THE PT COMMUNICATION RESOURCE

The Quiet Shift Series · Sovereign System Productions · 2026

SCRIPT 1

Patient Hasn't Done Their Exercises

The Situation

You ask: "How did the exercises go?" The patient looks at the floor. "I, uh, didn't get to them."

What Not to Say

"Why not?" — Sounds like an accusation. Triggers defensiveness.

What to Try Instead

Option 1 — Gentle curiosity:

"That happens. Was there something specific that got in the way this week?"

Option 2 — Problem-solving:

"Let us look at the exercise plan together. If it is too much or too hard to fit in, I want to know. We can adjust."

Option 3 — Reframe:

"The fact that you are telling me honestly means a lot. Let us figure out a version you can actually do. That is on me, not you."

Why This Works: Shame shuts people down. Curiosity opens them up. You are not judging. You are troubleshooting.

 **Research Note:** Self-Determination Theory (Ryan & Deci) shows that autonomy-supportive communication (vs. controlling) improves intrinsic motivation and adherence. "Let us figure this out together" supports autonomy. "Why didn't you do it?" undermines it.

Alternate Script — For the Defensive Patient:

"I am not here to judge you. I am here to help you. Something got in the way. Let us figure out what it was — and fix it."

SCRIPT 2

Patient in Pain and Afraid to Move

The Situation

The patient tenses up. "It hurts. I do not want to make it worse."

What Not to Say

"It is probably fine. Just try anyway." — Dismisses their fear.

What to Try Instead

Option 1 — Validate first:

"I hear you. Fear of making it worse is completely understandable. Let us find a version that does NOT make it worse."

Option 2 — Smallest step:

"What is the smallest, safest movement you feel willing to try today? Even a tiny one."

Option 3 — Education + permission:

"Pain does not always mean damage. Your alarm system might be overreacting. Let us test a very small movement and see how your body responds — we can stop anytime."

Why This Works: Fear is not overcome by logic alone. It is overcome by safety, titration, and small wins.

 **Research Note:** Graded exposure therapy (Vlaeyen & Linton) shows that fear reduction occurs through behavioral experiments, not persuasion. "Let us test a small movement and see what happens" is an experiment. "Trust me, it is fine" is persuasion.

Alternate Script — For the Highly Anxious Patient:

"We do not have to do the full movement today. We can just imagine doing it. We can watch me do it. We can do one tiny part. You are in control of how fast we go."

SCRIPT 3

Patient Is Frustrated with Slow Progress

The Situation

"I have been doing everything. Why am I not better?"

What Not to Say

"Be patient." — Invalidating.

What to Try Instead

Option 1 — Acknowledge + reframe:

"I hear your frustration. You are working hard. Let me show you the progress you might be missing." (Then show evidence — pain score down, reps up, etc.)

Option 2 — The stairs analogy:

"Progress often looks like stairs, not a ramp. Flat periods, then jumps. You might be on a flat part right now. That is normal. The jump is coming."

Option 3 — Adjust the plan:

"If you are doing everything and not seeing change, that is my signal to adjust something. Let me think overnight and come back with a new idea."

Why This Works: You validate their effort, show hidden progress, and own the need to adjust — which builds trust.

Alternate Script — For the Discouraged Patient:

"You are not failing. Your body is just taking longer than you hoped. That is not the same as failure. Let us look at the data together."

SCRIPT 4

Patient Compares to Someone Who Recovered Faster

The Situation

"My friend had the same thing and was better in three weeks. Why am I not?"

What Not to Say

"Every body is different." — True. Also dismissive if not accompanied by explanation.

What to Try Instead

Option 1 — Normalize + explain:

"That is frustrating. Here is what we know: age, starting fitness, tissue involvement, and even stress affect recovery speed. You are not your friend. Your body is on its own timeline."

Option 2 — Redirect to what you control:

"I cannot change your starting point. But I can help you maximize what you do now. Let us focus on what moves the needle for you."

Option 3 — Compassion + challenge:

"I hear the frustration. Comparing usually makes us feel worse. Can we agree to compare you to last week's you instead of someone else?"

Why This Works: Comparison is a trap. You redirect attention to what is measurable and controllable.

SCRIPT 5

Patient Thinks Rest Is Better Than Movement

The Situation

"I am just going to rest until it feels better."

What Not to Say

"Rest is bad for you." — Oversimplified. Triggers resistance.

What to Try Instead

Option 1 — Explain the risk:

"Complete rest for more than a few days can actually weaken muscles and stiffen joints. I am not saying push through pain. I am saying find the smallest amount of gentle movement that feels okay."

Option 2 — The Goldilocks frame:

"There is a 'just right' amount of movement — not too much, not too little. Let us find yours."

Option 3 — Permission + boundary:

"If you need a few days of rest, take them. But let us put an end date on it. After three days, we start very small — even sitting and standing a few times counts."

Why This Works: You validate rest as a short-term strategy while preventing rest from becoming avoidance.

 **Research Note:** Bed rest literature shows that immobilization for as little as 7 days causes measurable muscle atrophy and joint stiffness. Movement — even minimal — preserves function.

BONUS

HEP Adherence Cheat Sheet

Dosage, frequency, and progression — backed by research

Starting Point (Most Patients)

VARIABLE	RECOMMENDATION
Number of exercises	2-4
Frequency	3-5 days per week
Sets	1-2
Reps	8-12
Hold time (static)	10-30 seconds
Rest between sets	30-60 seconds

When to Progress

Patient completes prescribed frequency for 2 weeks consistently • Pain with exercise is <3/10 and settles quickly • Patient reports exercises feel "too easy"

How to Progress (Choose ONE at a time)

Add reps (12 → 15)	Add sets (2 → 3)
Add resistance (lighter → heavier band)	Add hold time (10 sec → 20 sec)
Add complexity (two planes of motion)	Add frequency (4 days → 5 days)

When to Regress

Pain with exercise >4/10 that persists after session • Patient consistently misses sessions (plan is too hard) • Poor form despite cueing



Moelands et al. (2025): Higher prescribed frequency and dosage of home exercises significantly lowers patient adherence. Sometimes less is more.

THIS IS JUST THE BEGINNING

You Just Got 5 Scripts.

The Full Book Has 35.

Inside **The PT Communication Resource**, you will also find:

- 25 Core Explanations — warm, simple versions of what you already know
- 35 Session Scripts — the largest section for a reason
- 15 Quick Reference Sheets — print-and-go for your desk or pocket
- 30 Q&As — real questions from PTs, answered with research and honesty
- 12 Companion Tools — templates, reflection logs, and checklists
- 12 Burnout Protection Tools — for you, not for patients

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